

HOUSE No. 2378

The Commonwealth of Massachusetts

PRESENTED BY:

Nicholas A. Boldyga

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act to increase patient access to certain health care services.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	DATE ADDED:
<i>Nicholas A. Boldyga</i>	<i>3rd Hampden</i>	<i>1/17/2025</i>

HOUSE No. 2378

By Representative Boldyga of Southwick, a petition (accompanied by bill, House, No. 2378) of
Nicholas A. Boldyga relative to patient access to certain health care services. Public Health.

[SIMILAR MATTER FILED IN PREVIOUS SESSION
SEE HOUSE, NO. 2139 OF 2023-2024.]

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act to increase patient access to certain health care services.

*Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority
of the same, as follows:*

1 The following portions of Section 25B of chapter 111 of the General Laws are hereby
2 amended to read as follows:

3 “Health care facility”, a hospital or clinic, as defined in section fifty-two; a long-term
4 care facility, a convalescent or nursing home, a rest home or a charitable home for the aged, as
5 defined in section seventy-one; a clinical laboratory subject to licensing under chapter one
6 hundred and eleven D, a public medical institution, which is any medical institution, and, after
7 December first, nineteen hundred and seventy-two, any institution for the developmentally
8 disabled or mentally ill, supported in whole or in part by public funds, staffed by professional,
9 medical and nursing personnel and providing medical care, in accordance with standards
10 established through licensing, approval or certification for participation in the programs

administered under Titles 18 and 19 of the Federal Social Security Act, by the department; and any part of such facilities; provided, however, that “health care facility” shall not include a facility operated by and for persons who rely exclusively upon treatment by spiritual means through prayer for healing, in accordance with the creed or tenets of a church or religious denomination and in which health care by or under the supervision of doctors of medicine, osteopathy, or dentistry is not provided; nor shall “health care facility” include ambulatory surgical centers.

“New technology”, equipment as defined by the department, or a service, as defined by the department, which for reasons of quality, access or cost is determined to be new technology by the department; provided, however, that computerized tomography and any equipment widely utilized as standard diagnostic treatment or therapeutic technology shall not be considered new technology and provided further that air ambulance service shall not be considered a new service.

“Substantial change in services”, shall mean: (1)(a) with regard to acute-care hospitals only, the addition or expansion of, or conversion to, innovative service regardless of whether an expenditure minimum is exceeded; (b) for any acute-care hospital, the addition or expansion of, or conversion to any services which may be provided by facilities which are not acute-care hospitals; except that conversions of acute-care services to skilled nursing, rehabilitation, acute psychiatric, and substance abuse services located in an underbedded areas shall be determined by criteria developed by the department in consultation with the department of elder affairs, department of mental health, the Massachusetts federation of nursing homes, the Massachusetts hospital association and other interested parties, and that no such conversion shall occur until the department has certified in writing the conversions meet the criteria established. The department

shall promulgate regulations to implement the provisions of said criteria for underbedded areas including, but not limited to medicaid access, and regulations to define criteria for reconversion; and (2) for any health care facility other than an acute-care hospital (a) the addition of a service which entails annual operating costs in excess of the expenditure minimum determined pursuant to this section; (b) any increase in bed capacity of more than twelve beds; (c) the addition or expansion of, or conversion to an innovative service regardless of whether an expenditure minimum is exceeded; (d) provided, however, that no decrease in the level of a service that, pursuant to department regulations, may be offered by a nursing, convalescent, or rest home which does not involve a capital expenditure in excess of eight hundred thousand dollars shall be subject to the provisions of sections twenty-five C to twenty-five G, inclusive; (e) provided, further, that an increase in staff by itself shall not be defined by the department to constitute a substantial change in service unless said increase in staff will result in an addition to annual operating costs which exceeds the expenditure minimum determined pursuant to this section. Notwithstanding any other provisions to the contrary, a change of service concerned solely with outpatient services other than ambulatory surgery, not otherwise defined as innovative services, shall not be defined by the department to constitute a substantial change of service.